Arolygiaeth Gofal Cymru
Care Inspectorate Wales

# Inspection Report

## 20 Francis Avenue

20 Francis Avenue, Rhos On Sea,  Colwyn Bay, LL28 4DW

01492545370

www.coeddu.com

**The inspection visit took place on 29/06/2026**

## Service Information:

| | |
|---|---|
| Operated by: | Coed Du Hall Ltd |
| Care Type: | Care Home Service<br>Adults Without Nursing |
| Provision for: | Provision for learning disability, Provision for mental health |
| Registered places: | 5 |
| Main language(s): | English |
| Promotion of Welsh language and culture: | The provider makes an effort to promote the use of the Welsh language and culture or is working towards a bilingual service. |

## Ratings:

| | |
|---|---|
| Well-being | Excellent |
| Care & Support | Excellent |
| Environment | Good |
| Leadership & Management | Good |

## Summary:

20 Frances Avenue is a small home that supports young adults with mental health needs, who aspire to live independently.

Well-being is excellent because everyone is treated with dignity and respect and are actively and creatively supported to achieve their wellbeing outcomes. They have as much control over their lives as possible and experience a sense of calm, freedom, stability and security.

Care and support is excellent because people experience very high quality care and support; the manager completes in-depth, effective assessments of each person's needs. People build very strong relationships with the manager and staff, and compatibility is carefully considered so they get along with each other. People have significant involvement in their assessments, care plans and reviews. They achieve their outcomes and some have gone on to live independently.

The environment is good because it meets people's needs. Large bedrooms and quality fixtures and fittings support people's wellbeing; they feel they have the space and privacy they need to

experience a level of independence while enjoying the security of staff support.

Leadership is good, and we saw some excellent practice. There are a range of measures in place to ensure good practice and continued compliance with regulations. The RI (responsible individual for the home) completes thorough analysis of the service and knows in great depth what is working well and where improvements could be made.

# Findings:

## Well-being — Excellent

People live healthily and safely with control over their lives. Everyone is treated with dignity and respect and are actively supported to achieve their wellbeing outcomes. They have as much control over their lives as possible, partaking in activities they have chosen and at their own pace. Care is highly person centred as people are actively involved in decisions that affect them, coproducing their plans and ensuring their voices are respected. Information in plans is highly tailored to people's individual preferences, likes and dislikes and their aspirations. They receive support for their health needs, both from external health care and from the services own health practitioners who have the expertise required. People shared how happy they are to live here, to have their own space in which their privacy is respected, and the pace of life is calm.

People are safe and protected from abuse because they know how to raise concerns. Their rights are upheld; being supported to access the right health care, welfare benefits, finding homes and using the community. People are free to come and go as they please and are kept safe with appropriate security of the building at night. They describe a consistent culture of openness without judgement and feel they can speak with staff freely. Positive risk taking is fully supported so people learn to budget, shop and cook, enhancing their independence skills safely and with guidance.

Safe and healthy relationships are cultivated. Opportunities to socialise in the community are taken up, such as going to local cafes, gyms and clubs; home stays with family are encouraged where appropriate. People enjoy their gaming with friends and have access to IT and their phones to contact friends and family. Each person has a twelve-week assessment period on admission into the home to ensure it is the right place for them; strong friendships with each other are made more possible because compatibility is key factor.

The accommodation supports people's wellbeing and outcomes. We spoke to three people who expressed great satisfaction with their bedrooms; they are large and provide them with comfortable, private space to sleep, watch television or follow other hobbies and interests, and shower in their large en-suite facilities. There is a choice of communal rooms to lounge in and free access to the kitchen, where people can store food they have shopped for and cook their own meals. Promoting independence is a key aim of the service.

## Care & Support — Excellent

People receive excellent support and achieve their personal outcomes. Care is designed with the person receiving it, so it is personalised and planned around their wishes, needs, strengths and aspirations. We learned how two people have moved on to independent living since the last inspection. We spoke with one of those people who shared their highly positive experience living in the home. They said, *'it gave me time to breathe and bounce back'* and '*support was always at my pace – no pressure. It was nice there, a good place to live'.* Another person said *'staff are great. I am accountable but have my freedom. I feel good because I'm doing well'.* Progress towards outcomes is measured frequently throughout the year when new goals are also considered. Step by step plans are put in place, breaking big outcomes into incremental goals to make them more achievable. We saw evidence of people achieving their outcomes such as attending college, learning to drive, learning to manage their own medication, and keeping fit. The service is part of a larger organisation which employs its own health professionals in psychology, psychiatry, occupational therapy and nursing, enabling early intervention.

In addition to the service's culture of openness, enabling honest conversations, people are also protected by safeguarding policies and procedures. All staff are trained in safeguarding, so they know what they need to do if they have a concern. People have access to advocacy services if they need someone else to speak on their behalf. Everyone is supported to access facilities in the community that promote their wellbeing, including leisure facilities and health appointments, and in house expertise. Oral care is promoted; staff are trained and each person has a mouth care plan; they are supported to attend dental and GP appointments.

People's medication is safely managed. It is safely and securely stored, recorded and administered. Staff are trained to administer medication and their competency is checked. Most people living in the service are supported to be independent with their medication, within risk management strategies, which include breaking the activity down into steps and monitoring incremental progress towards complete independence. Every person has a locked cabinet in their room in which medication can be stored, and the checks and arrangements in place help ensure nothing is missed. The manager completes a monthly medication audit to check medication processes are compliant.

## Environment — Good

The home provides an environment conducive to supporting people's outcomes. Bedrooms are spacious so people feel they can spend a lot of time there. Each person has a comfortable bed, quality furniture for storing their clothes, shelving and space for gaming gear and a television. We saw a sofa in one bedroom and armchairs in others. Each room has a large ensuite including a wet room. People shared how pleased they are with their room; they feel the home offers freedom, stability and security and the environment is a contributing factor. One person said, *'I love everything about my room'*. He compared his room to having *'my own flat'* and expressed how it made him feel independent. There are two large, comfortable, well lit lounges and other communal spaces, including a games room. People help themselves in the equipped kitchen, where there is ample space for meal preparation and dining. The laundry facilities allow people to take care of their own laundry, with support if required. Established routines ensure bedrooms are cleaned weekly and laundry is taken care of. People let staff know when they are going out but are free to do so whenever they wish. The garden is maintained and there is seating and shelter for those who smoke.

The provider employs a maintenance person to oversee homes in the organisation. There is currently no separate log of repairs and maintenance work for Francis Avenue, but the manager assured us they can raise any issues for repair, and they are dealt with promptly. We found the home to be a positive, bright environment. It is beautifully furnished with an ornate wooden staircase and quality furniture throughout. Some minor paintwork and floor scratches had already been identified by the manager and are being addressed. Adhesive privacy film on some bedroom windows blocks the view. We discussed other measures that can be taken to afford privacy without blocking a view and the manager is looking at this.

Records show compliance with safety checks in relation to water, electricity, gas, fire, and equipment. Routine inhouse checks are completed to ensure fire safety and a record is kept. The home holds the highest score of level 5 from the Food Standards Agency for food safety practices. Infection control audits completed.

## Leadership & Management — Good

People are supported to achieve their outcomes because the provider has effective governance arrangements in place. They have good oversight of the service including the staff and people living here. Records evidence the RI visits the home at least every three months, to speak with staff and people using the service, check records, policies and processes and ensure compliance. They complete audits to ensure compliance, and check audits carried out by the manager such as medication audits, infection prevention checks and care record audits. Staff told us they know the RI well and have confidence in their willingness to respond to queries and keep staff updated. People told us they also find the RI very approachable, and that they listen and understand their challenges and outcomes. The RI uses a range of evidence to gain feedback such as one-to-one meetings with people using the service and the staff, as well as surveys. Feedback informs the bi-annual review of the quality of care provided. We reviewed associated reports which are comprehensive, evaluative and show year on year progress on desired improvements and developments. We saw actions from previous reviews, RI visits, and audits are consistently reviewed to assess progress and identify any further priorities.

Records show staff are safely recruited using established procedures for vetting and assessing suitability, including checking references and criminal records data. We spoke with staff who confirm they feel supported. They can meet with management every three months for one to one support, during which they discuss work practices, and future career goals. Staff can access the service's employment assistance programme, including health and well-being advice. Staff told us how much they enjoy working in Francis Avenue; *'I really like getting up in the morning and coming to work'*. They feel well trained and records show a good range of mandatory training and other training relevant to individuals they support. The service uses *What's the Message* training to understand positive behaviour support and how behaviour is a form of communication. The service is proactive in seeking ways to develop and has invested in technology that will reduce a reliance on paper and make records more accessible and easier to keep.

# Areas identified for improvement

Where we identify **Areas for Improvement** but we have not found outcomes for people to be at immediate or significant risk, we discuss these with the provider. We expect the provider to take action and we will follow this up at the next inspection.

Where we find outcomes for people **require significant improvement** and/or there is risk to people's well-being we identify areas for **Priority Action**. In these circumstances we issue a Priority Action Notice(s) to the Provider, and they must take immediate steps to make improvements. We will inspect again within six months to check improvements have been made and outcomes for people have improved.

**CIW has no areas for improvement identified following this inspection.**

**CIW has not issued any Priority action notices following this inspection.**

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